

Clinical Decision-Support: Comprehensive Genetic & Dermatological Dossier

PHYSICIAN & MEDICAL GENETICS EDITION | Record ID: MM-TCGA-D-A9B4 | Model: Hybrid-XGB-AdaBoost-VQC-Derm-v2.0

Patient Demographics	Age: 63.0 yrs Biological Sex: Male Detected Phototype: Type II	Tri-Model Consensus Risk	76% (HIGH RISK)
Dual Boosting + VQC	XGBoost (0.45) + AdaBoost (0.25) + 4-Qubit VQC (0.30)	Epistemic Uncertainty	Confidence: High ($\sigma = \pm 0.0717$)

1. Detailed Molecular & Genetic Deficiencies Analysis

Genetic Pathway / Defect	Status / Score	Classification	Clinical Oncologic Implications
TP53 Tumor Suppressor (17p13.1 DNA Binding)	Score: 0.78 / 1.0	PATHOGENIC	Loss of apoptotic checkpoint control, impaired p21/CDKN1A induction, elevated genomic destabilization.
BRCA1 / BRCA2 (HR Repair Pathway)	Absent (0.0)	Intact	Homologous recombination double-strand break repair failure; high sensitivity to PARP inhibition & platinum agents.
Tumor Mutational Burden (Somatic Density)	28.5 mut/Mb	HIGH (TMB-H)	Elevated somatic neoantigen expression. Biomarker for immune checkpoint inhibitor response (Anti-PD-1/PD-L1).
hs-CRP / Inflammation (Microenvironment)	5.4 mg/L	ELEVATED	Sustained pro-inflammatory cytokines (IL-6 / TNF-alpha) facilitating cutaneous angiogenesis & immune evasion.
Hereditary Pedigree (First-Degree Relatives)	Confirmed (1.0)	POSITIVE	Germline susceptibility inheritance profile. Multi-generational screening indicated.

2. Dual Boosting (XGBoost + AdaBoost) & Parameterized Quantum VQC Ensemble

Inference Engine	Estimated Probability	Weight	Algorithm Architecture & Role
XGBoost (Gradient Boosted Trees)	77%	45%	Depth-6 gradient boosting over combined 17-dimensional cutaneous & genomic vector.
AdaBoost (Adaptive Stumps)	64%	25%	Sequential decision stumps emphasizing borderline cases with ambiguous melanin-genomic interplay.
Quantum VQC (Qiskit Aer / PennyLane)	74%	30%	4-qubit parameterized variational circuit with angle embedding exploring non-linear Hilbert space correlations.
Hybrid Calibrated Consensus	76%	100%	Calibrated consensus with discordance variance boundary (± 0.208). Stratified to HIGH RISK.

3. Model Risk Spectrum Across All Skin Phototypes (Fitzpatrick Types I–VI)

Simulates the patient's exact genetic mutations across all skin pigmentation categories to evaluate cutaneous susceptibility:

Fitzpatrick Phototype	Optical ITA / Melanin	XGBoost	AdaBoost	Hybrid Consensus	Stratification Tier
Type I (Very Light) [PATIENT PHENOTYPE]	ITA: 78.0° Mel: 3.5	99%	92%	88%	Very High Risk
Type II (Light) [PATIENT PHENOTYPE]	ITA: 52.0° Mel: 14.0	99%	90%	87%	Very High Risk

Type III (Intermediate)	ITA: 35.0° Mel: 22.0	99%	89%	86%	Very High Risk
Type IV (Tan / Olive)	ITA: 18.0° Mel: 30.0	97%	88%	85%	Very High Risk
Type V (Brown)	ITA: -12.0° Mel: 46.0	94%	85%	83%	Very High Risk
Type VI (Dark / Deeply Pigmented)	ITA: -62.0° Mel: 72.0	90%	81%	79%	High Risk

4. Actionable Clinical Guidance & Triage Protocol

Secondary lifestyle assessment and scheduled follow-up testing advised within 60 days.

MANDATORY MEDICAL DISCLAIMER: AI-generated decision-support output. Not a final medical diagnosis. Final clinical decision remains with a qualified healthcare professional.